



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متدد

إسم المريض رباغياً : مفيد اسامه عمر محمد

العنوان : مساكن الاميريه

الحي / المدينة : الاميريه

المحافظة : القاهرة

القديمة ب 18 م3

تاريخ تسجيل الإستمارة : 04-10-2026

رقم البطاقة : 27810010106846 رقم التليفون : 01201509336

العمر(تاريخ الميلاد) : 01-10-1978 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع: (-----)

المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمى وتلطيفى [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة



الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبى للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / أ.د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبى المتخصص

Name: مفيدة اسامه عمر محمد
National ID: 27810010106846



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
سلطنة كويت

مفيد
اسماء عمر محمد

مسكن الاميرية القديمة ١٨
الاميرية - القاهرة

٢ ٧٨ ١٠ ٠١ ٠١ ٠٦ ٨ ٤ ٦

KB815

٢٠٢٣/٠٧ ٢٧٨١٠٠١٠١٠٦٨٤٦

بدون عمل

أنثى مسجلة متروكة

علاء الدين زكى سيد احمد سعد

البطاقة مازية حتى ٢٠٣٠/٠٧/٢٥

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مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السلطة المصرية
للصحة
مصر

MDT FORM 2015/2025 G M

Name	مفيدة سلامة محمد	MRN	٢٠٢٥ - ١٤٦٤ -
Age	43 years	ID	٢٧٨١٠٠١٠٦٨٤٧
Menopausal status	premenopausal	Phone	01090823767
NRS SCORE		Marital status	married with 5 children
BMI		Family history	-ve
CO morbidity	DM, HTN	REASON OF MDT	

Clinical Data

BX: IUC GII

ER: 8/8	PR: 7/8	HER 2: 2	KI67: 50%
-ve for amplification			

R

L

Sonomammography:

Mets Workup:

2023 ⇒ Rt Breast Cancer

2/2023 ⇒ few lytic lesions eth rib, iliac bone, moderate pleural effusion.

ultrasound abdomen regarding hepatic focal lesion ⇒ Hemangioma.

Recommendations:

PET 8/8/2024 ⇒ stationary course of Rt breast lesion

- Rt pleural thickening.

- metabolic resolution of active osseous lesions.

TNM, pleura

MDT OUTCOME & PLAN

Fasbider, Ibrance, Zoladex

→ Zometa / 6 months after dental assessment.
For palliative Rth on Rt Breast

* New CT (AP) ⇒ baseline

SIGNATURE

٢٠٢٥ / ١٠ / ٤

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

M. Kameh

Name: مفيدة اسامه عمر محمد		
MR. : 3279		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها		
MBC ON IBRANCE *AROMAZINE TLC 0.6 POST PONE ONE WEEK 47 years old femae pt with rt breast cancer idc gii er 8/8 pr 7/8 HER2+2 NOT AMPLIFIED KI6750% DENOVO MBC TO BONE 1/2023 RT BREAST CANCER ON AI TILL 2/2025 THEN PD IBRANCE SINCE 11/2025 ET 2/2025 FEW LYTIC LESIONS FOR FASLODEX+ZOLADEX +AROMASINE THEN PD PET CT 8/8/2024 SD OF RT BREAST CANCER RT PLEURAL EFFUSION BONY LESION BONE SCAN 4/2/025 NEWLY DEVELOPED CEVICAL DOSRAL SPINE LESION ON IBRANCE125+AROMASINE +ZOLADEX AROMASINEE+ZOLADEX FROM OUTSIDE C 4 IBRANCE 125 FOR CT CAP BS 19\4\2026 >> REGRESSIVE DISEASE CT CAP 24\4\2026 >> PENDING REPORT TODAY C5 IBRANCE (LABS ACCEPTED) for ibrance 125 mg+suppotrive tt patient didnt dispense last time CBC NEUTROPENIC ANC:800 ..FOR REST AND FOLLOW UP CBC AFTER 1 WEEK TLC /ANC 3.2 /1.9 ACCEPTED FOR C6 IBRANCE + AROMASINE + SUP TTT pt receives aromasine from outside (egyptair hospital) and ibrance from GRI hb 10 tlc 2 anc 1.1 plt 158 for aromasine		
تحريراً في ,		
	التاريخ :	19-09-2026
	الوقت :	نهى سعيد علي محمد
		الطبيب المعالج :
		يعتمد ,

Report Number: m1940

Female : النوع
القاهرة : العنوان
02/12/2025 11:58 : تاريخ استلام العينة
02/12/2025 13:01 : تاريخ التقرير

اسم المريض :
السن :
تاريخ اخذ العينة :
طلب التحليل :
عناية :
مفيدة اسامه عمر
45 Y
نقدى
نقدى

Primary Sample :

Three immunostained slides labelled Egypt Air , 58-23 , ER , PR and Ki67

Test :

ER & PR & Ki67 (ER & PR: Harvey scoring system)

Comment :

Technique :

Result :

ER: strong nuclear staining of 95 % of tumour cells
PR: strong nuclear staining of 90 % of tumour cells
Ki67: positive nuclear staining of > 20 % of tumour cells

Referral Lab :

Comment :

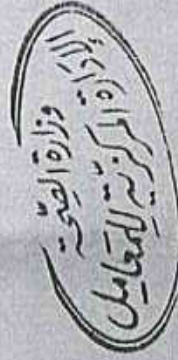
Conclusion :

ER: POSITIVE (SCORE 8/8)
PR: POSITIVE (SCORE 8/8)
#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS > 20 %

Pathologist:

د/ليناس رمزي

Lab Manager: د/تريزا ماهر





ImmunohistoChemistry report

Report Number: m.1940

Female :	النوع	مفيدة اسامه عمر	اسم المريض
القاهرة :	العنوان	45 Y	المسن
02/12/2025 12:50 :	تاريخ استلام العينة		تاريخ اخذ العينة
02/12/2025 13:03 :	تاريخ التقرير	نقدى	طالب التحليل
		نقدى	عناية

Primary Sample :

one immunostained slide labelled DR,Elia Lab , Mofida Osman , 555273 , Her2 Sish

Test :

HER2 by SISH

Comment :

Technique :

Result :

The ratio of Her 2 / chr 17 is less than 2

Referral lab :

Comment :

Conclusion :

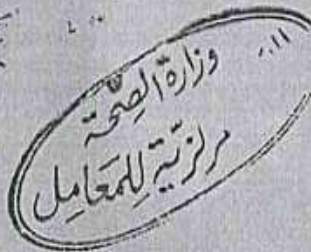
Her 2 Status by SISH (Dual Ish Technique): Negative (not amplified Her 2 / chr 17 < 2) (HER2 LOW)

Pathologist:

د/أيناس رمزي

Lab Manager:

د/تريزا ماهر



Pat. Name: MOFIDA OSAMA OMAR

Pat. Code:

Study Date 04/05/2025

Rad Dept ID: 5177869

CT SCAN OF THE CHEST

HISTORY: A known case with right breast cancer on follow up. Compared to the last available previous study dated 28/01/2025, the current study reveals:

TECHNIQUE:

Axial contiguous thin cuts were taken through the chest after IV contrast injection.

FINDINGS:

- Progressive course is noted regarding the size of the previously noted large irregular right breast malignant looking mass lesion currently = 50 x 48 mm compared to 39 x 38 mm in average on the previous study.
- A stationary course is noted regarding the size of the irregular suspicious malignant looking satellite lesions with the largest currently = 39 x 30 mm.
- Progressive course regarding the size of the suspicious looking right axillary lymphadenopathies with the largest currently = 31 x 21 mm compared to 20 x 20 mm.
- Complete resolution of the previously noted right upper lobe apical solid pulmonary nodule as well as few right upper lobe patchy areas of ground glass opacities.
- A stationary course of mild likely encysted right side pleural effusion.
- A progressive course regarding The size of the right paratracheal lymphadenopathies with the largest currently = 24 x 21 mm.
- Stationary course is noted regarding the previously noted abnormal right breast skin thickening and edema.
- Still noted few bilateral pulmonary atelectatic bands.
- No evidence of abnormal masses, other nodules or consolidative patches.
- Normal CT appearance of the mediastinum vessels.
- No other sizable mediastinal or hilar adenopathy.
- Patent tracheobronchial tree with no abnormal bronchial dilatation.
- No left side pleural or pericardial effusion.
- Cardiomegaly is still noted.

IMPRESSION:

- A follow up study reveals progressive course regarding the size of the right breast malignant looking mass lesions as well as the size of the right axillary malignant looking lymphadenopathies and the right paratracheal lymphadenopathies.
- A stationary course regarding the right breast satellite lesions and the encysted right side pleural effusion
- Complete resolution of the right upper lobe pulmonary changes.
- Stationary courses regarding the pulmonary atelectatic bands and the cardiomegaly.

Much obliged,

Dr. Ahmed M. Osman, MD

Professor of Radiodiagnosis, Ain Shams University

Pat. Name: MOFIDA OSAMA OMAR
Study Date 15-02-2024

Pat. Code:
Rad Dept ID:
QC1 517869

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION

- A known case of right breast cancer. No available previous studies

Findings:

- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- The right breast shows diffuse skin thickening with an upper outer speculated irregular mass lesion measuring 26 x 28 mm with surrounding two satellite lesion measuring 21 x 16 mm and 23.3x 19 mm. Few suspicious looking right axillary lymph nodes, the largest is measuring 10 x 9.5 mm for dedicated sonomammographic assessment

Opinion:

- A known case of right breast cancer. No available previous studies
- Diffuse right breast skin thickening with an upper outer speculated irregular mass and few surrounding satellite lesions together with few suspicious looking right axillary lymph for dedicated sonomammographic assessment

Thank you for your referral

Yours truly,

Dr. Dalia El-Mossly, MD, FRCR

Consultant of Radiodiagnosis, Ain Shams University

Nuclear Medicine Report

Name: السيدة / مفضلة اسامة عمر	Age: 46 yrs
Date : 4 / 2 / 2025	Code: 01284324

Tc-99m MDP Bone Scan

In comparison to previous scan on 27/2/2024 (~ 1 yr)

- ❖ Present scan reveals newly developed foci of enhanced tracer uptake at cervico-dorsal spine, 12th dorsal vertebra and Lt sacro-iliac joint suspecting early bone metastatic deposits. Clinical and radiographic correlation is recommended.
- ❖ The rest of surveyed skeleton shows nearly the same tracer biodistribution as before with no abnormality suspected for destructive osteoblastic deposits.
- ❖ Osteoarthritic reactions are still suggested for shoulders. Bilateral trochanteric bursitis is also still impressed.
- ❖ Rt breast shadow is markedly enlarged in present scan with dense soft tissue count indicative of edema, to check clinically.
- ❖ Rt kidney is atrophied & ptosed with activity retention while Lt kidney shows compensatory hypertrophy.

Signature

Dr. Magdy Shaaban, MD

Magdy Shaaban



مستشفى
عين شمس

PATHOLOGY & CYTOLOGY REPORT			
Patient Name: مفيدة اسامه عمر		Sex: F	Age: 44 y
Referred by: Dr. Waleed Hetta		Dept.: Radiology	
Code: 01284324		Report No.: 23-58	

Specimen(s): 1. Core biopsy, axillary lymph node . 2. Core biopsy, breast mass

Clinical Data: Suspicious right breast mass – Axillary lymph nodes

Gross Description:

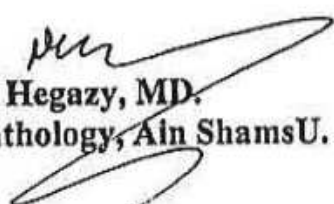
1. Core of soft tissue measuring 1 cm long. Totally submitted in 1 cassette.
2. Two cores of soft tissue; the longest is 1.5 cm. Totally submitted in 1 cassette.

Microscopic Description:

1. Sections reveal invasive nests and sheets of malignant epithelial cells exhibiting moderate anaplasia with poor attempts at tubular differentiation. No lymphoid tissue is included.
2. Sections reveal invasive nests, sheets and cords of malignant epithelial cells exhibiting moderate anaplasia with poor attempts at tubular differentiation. There is considerable mitotic activity. No in situ component is detectable.

Pathologic Diagnosis:

1. Core biopsy, axillary lymph node: Positive for carcinoma
2. Core biopsy, breast mass: Invasive duct carcinoma, grade II


Dr. Nedal Hegazy, MD.
Prof. of Pathology, Ain ShamsU.

Dr. Magda S. El Monayeri, MD.
Prof. of Pathology, Ain ShamsU.

Date: 15/1/2023



مركز
التشخيص

PATHOLOGY & CYTOLOGY REPORT			
Patient Name: مقيده اسامه عمر	Sex: F	Age: 44 Y	
Referred by: د. وليد حنه	Dept.: ICU	Report No.: 23-58	
Code: 01284324			

Immunohistochemistry Report

Specimen(s): Referred paraffin block (labeled as 58-23 (2))

Diagnosis: Right breast mass: Invasive duct carcinoma

Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by monoclonal antibodies and detection kit using Ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections were treated against ER, PR, HER-2 neu & ki67

Immunohistochemical Results:

- Estrogen Receptors: Strong positive staining of 90% of tumor cells. Allred score 8/8
- Progesterone Receptors: Moderately intense positive staining of 80% of tumor cells. Allred score 7/8
- HER-2 neu: Equivocal (score 2+), for FISH testing.
- Ki-67 proliferation index 50% of tumor cells

Dr. Nedal Hegazy, MD.
Prof. of Pathology, Ain ShamsU.

Dr. Magda S. El Monayeri, MD.
Prof. of Pathology, Ain ShamsU.

Date: 23/1/2023

Case Resume

مفيدة أسامة عمر

46 yrs, DM, premenopausal, P.S. (ECOG): 1

Presented on 1/2023 with locally advanced breast with peau d'orange

1/23: Breast biopsy + positive axillary biopsy : IDC , GII , ER(8/8) , PR(7/8) , HER2 +2 SISH -ve , KI 50%

CT chest mild pleural effusion

CT abdomen: HFL 1 cm >> proven benign by MRI

PET CT 2/23: RT breast lesions , RT axillary LNs , few lytic osseous lesions likely metastatic (ribs RT side , LV1 , Lt iliac bone) , moderate to marked RT pleural effusion , no liver lesions

Pleural effusion cytology: high atypical suspicious cells

3/23 FEMARA/ ZOLADEX with good clinical and radiological response >> resolution of pleural effusion and axillary LNs, SD breast

2/25 >>> CA15.3 133 vs 44 (8/24)

CT chest & PA compared to 2/24:

- PD of rt breast mass lesions as well as satellites
- De novo encysted RT pleural effusion as well as apical lung lobe nodule and RT para tracheal adenopathy
- De novo few round glass opacities
- SD HFL >> proven before benign by MRI
- Moderate hydronephrosis >> obstructing large renal ureteric stone
- SD of lt bulky ovary
- SD scattered bone lesions, dorsal, lumbar vertebrae, rt lower ribs

BONE SCAN: newly developed foci at cervicodorsal spine, 12th dorsal vertebrae, Lt. sacroiliac joint suspecting early bone mets

Committee decision on 13/2/25:

- FOR UROLOGY CONSULTATION ABOUT THE STONE TO AVOID RENAL AFFECTION
- CONTINUE SAME TTT AS MINIMAL PD

Continued the same ttt...

CT Chest on 4/5/25 compared to 28/1/25:

- PD of breast lesion (50*48 mm vs. 39*38 mm)
- SD of satellite lesions, largest 39*30 mm
- PD of Rt. axillary LN's (31*21 mm vs. 20*20 mm)
- CR of Rt upper lung lobe apical nodule
- SD of mild encysted Rt. encysted pleural effusion
- PD of Rt. paratracheal LN's, largest 24*21 mm

CT abdomen & pelvis on 4/5/25 compared to 28/1/25:

- SD of the HFL, atrophic malrotated RT kidney, moderate hydronephrosis due to stone , SD of bone sclerotic lesions

Decision of the oncology committee at EgyptAir Hospital on 18/5/2025:

- For palliative RTH to the RT breast as skin nodules
- For Faslodex / CDK4/6 inhibitor + Zometa (will be received in HARMEL)

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مفيدة اسامه عمر محمد

Age : 44 Y : 4 M : 1 D

Referred By Prof. Dr. : محمد عبد الله

Clinical Diagnosis : Breast mass

Nature of Specimen : Referred block labelled as 58/23 (Core biopsies)

Receiving Date : 2023-02-02

Analysis ID : 722694

Gender : Female

Patient ID : 555273

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections prepared from the paraffin block were stained for HER 2 gene and chromosome 17 using Silver In Situ Hybridization (SISH) technique in Benchmark XT (Ventana).

Results :

The ratio of Her 2 signals to chromosome 17 signals in the nuclei of the tumour cells is 1.5 (less than 2) indicating absence of amplification of HER2 gene.
Average Her 2 copy nb/cell is 3.

Conclusion :

Referred block, HER 2 by SISH, NEGATIVE FOR HER 2 GENE AMPLIFICATION, RATIO IS 1.5.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak,

Shady Elia

Elia anis



This report is electronically signed , any abrasion or modification would render it void .

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٢٠٢٠



ERM036745744

تمت الموافقة على التسجيل
بطاقة تحقيق الشخصية

١٩٧٨/١١/١١ ٢٧٨١٠٠١٠١٠٢٨٤٦
KB8150799

ملقيه
اسمه عمر محمد
مسكن الاميرية القبيصة ١٨
الاميرية - القاهرة

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٢٠٢٠

١٢٢٠/٧ ٢٧٨١٠٠١٠١٠٢٨٤٦
بدون عمل

مسترجعة
علاء الدين زكي سيد احمد
١٢٢٠/٧/٢٥

بطاقة سارية حتى

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EGYPTAIR HOSPITAL
مستشفى مصر للطيران

شركة مصر للطيران للخدمات الطبية

Pat. Name: MOFIEDAH OSSAMA OMAR MOHAMED	Pat. Code: 27810010106846
Study Date 28/09/2026	Rad Dept ID: 1284324

CT SCAN OF THE ABDOMEN AND PELVIS

HISTORY: A known case with right breast cancer on follow up. Compared to the last available previous study dated 23/04/2026, the current study reveals:

TECHNIQUE:

Axial contiguous cuts were taken through the abdomen and pelvis with oral and IV contrast injection.

FINDINGS:

- The liver is enlarged in size displaying diffuse decreased attenuation pattern, denoting diffuse fatty infiltration. Stationary course is noted regarding the size of the hepatic segment VIII focal lesion still = 16 x 15 mm, no other hepatic focal lesions noted.
- No extra or intrahepatic biliary dilatation.
- The PV: is of average diameter with normal contrast opacification with no CT evidence of internal thrombosis.
- The spleen is of average size displaying normal and homogeneous CT appearance.
- The pancreas shows normal C.T. appearance.
- The right kidney is still atrophic displaying small size being malrotated with thinned renal parenchyma still showing small size stones as well as moderate hydronephrosis due to a large renal pelvic / upper ureteric stone = 17 x 16 mm.
- The left kidney is seen in its normal anatomical site displaying average size and normal shape (malrotated). No evidence of abnormal masses or backpressure changes.
- Normal size and CT density of the adrenal glands.
- No sizable abdominal or pelvic adenopathy.
- No evidence of ascites.
- Normal appearance of the urinary bladder with normal wall thickness and no calculi, masses or diverticula.
- Normal CT appearance of the uterus and both adnexal regions apart from stationary course regarding the bulky size of the left ovary.
- Normal CT appearance of the scanned bowel loops.
- No pelvic masses or fluid collection.
- With stationary course, bone window reveals extensive widespread lytic and sclerotic lesions involving the whole scanned spine, sternum, pelvic bone, and proximal parts of the femoral bones.

IMPRESSION:

- A follow up study reveals stationary course regarding the size of the hepatic segment VIII hypodense focal lesion.
- Stationary course regarding the fatty hepatomegaly.
- Stationary course regarding the Small-sized right kidney, being malrotated with an obstructing pelvic stone and a subsequent moderate hydronephrosis and thinned renal parenchyma.
- Stationary course regarding the widespread lytic and sclerotic lesions involving the whole scanned spine, sternum, pelvic bone and proximal parts of the femoral bones, consistent with bone metastases.

Much obliged,

Dr. Ahmed M. Osman, MD

Professor of Radiodiagnosis, Ain Shams University

Electronically Signed By: Ahmed Osman
28/09/2026 19:45

Pat. Name: MO'ETADAH OSSAMA OMAR MOHAMMED	Pat. Code: 27810010106846
Study Date 28/09/2026	Rad Dept ID: 1284324

CT SCAN OF THE CHEST

HISTORY: A known case with right breast cancer on follow up. Compared to the last available previous study dated 23/04/2026, the current study reveals:

TECHNIQUE:

Axial contiguous cuts were taken through the abdomen and pelvis with oral and IV contrast injection.

FINDINGS:

- Stationary course regarding the abdominal irregular right-sided circumferential pleural thickening, reaching the maximum at the para-vertebral region (20 mm) associated with minimal right-sided pleural effusion.
- Stationary course regarding the right upper lobe anterior and right middle lobe peripheral reticular infiltration and fibrosis with underlying fibro-atelectatic bands.
- Progressive course regarding the right lower lobe scattered atelectatic bands and septal thickening.
- Progressive course regarding the right breast skin thickening and edema with an irregular speculated soft tissue mass lesion currently = 45 x 38 mm compared to 50 x 48 mm in average on the previous study.
- Progressive course regarding a few small-sized right paratracheal and subcarinal lymphadenopathies, with the largest = 12 x 10 mm.
- No evidence of abnormal masses, nodules or consolidative patches.
- Normal CT appearance of the mediastinum vessels.
- Patent tracheobronchial tree with no abdominal bronchial dilatation.
- No left-sided pleural or pericardial effusion.
- Normal cardiac borders and chambers.

IMPRESSION:

- Allow up study reveals progressive course regarding the size of the right breast malignant looking mass lesions as well as the size of the right axillary malignant looking lymphadenopathies and the right paratracheal lymphadenopathies.
- A stationary course regarding the right breast satellite lesions and encysted right side plural effusion.
- Stationary course regarding the pulmonary atelectatic bands.
- Complete resolution of the right upper lobe pulmonary changes.

Much obliged,
Dr. Ahmed M. Osman, MD
Professor of Radiodiagnosis, Ain Shams University

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Nuclear Medicine Report

Name: السيد / مهيبة اسامة عمر	Age: 48 yrs
Date : 16 / 9 / 2026	Code: 01284324

Tc-99m MDP Bone Scan

In comparison to previous scan on 19/4/2026 (5 months)

❖ Metastatic bone deposits are re-evaluated at following skeletal sites:

1. Skull calvarium → foci are increased in size & number → new lesions
2. Manubrium sterni → enhanced → increase in size and density
3. Cervical & upper dorsal spines → re-enhanced
4. Lt shoulder → decrease in count → improved
5. Inferior angle of Lt scapula → same
6. Multiple rib lesions on both sides posteriorly → new
7. Mid dorsal vertebrae → new
8. L2 & L3 → the same while L4 & L5 → new
9. Both SI joints → enhanced with higher count density
10. Both acetabulums, Rt ischium & Lt ASIS → new
11. Rt femoral neck → enhanced with increase in size
12. Rt femoral shaft → faint new foci

- ❖ Progressive disease course is impressed by enhancement in most of lesions and appearance of new sites. Clinical correlation is recommended.
- ❖ Rt ankle arthritis is observed in present scan, to revise clinical data.
- ❖ Rt breast edema is still massive with dense soft tissue count.
- ❖ Rt kidney is atrophied & ptosed.

Signature

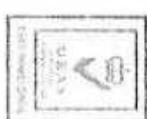
Dr. Magdy Shaaban, MD

Magdy Shaaban

16 September 2026, 11:41:49 PM



معامل روبيال لاب



Patient No.	23211969-7	Sex	Female	Age	46 Y
Accession No.	7287735	Reg. Date	22/09/2026 08:14		
Name	الاستاذة / منيرة لسانة عمر	Collection Date	22/09/2026 08:16		
Referred By	self / ا.ل	Reporting Date	22/09/2026 16:55		

Complete Blood Picture (EDTA)

Test	Result	Unit	Ref. Range
Red cell count	3.0	L $\times 10^6/\mu\text{L}$	3.8 - 4.8
Haemoglobin	10.4	g/dL	12.0 - 15.0
Haematocrit (HCT)	31.5	%	36.0 - 46.0
MCV	104	fL	83 - 101
MCH	34	pg	27 - 32
MCHC	33	g/dL	32 - 35
RDW-CV	15.5	%	11.6 - 14.0
Platelet count	279	$\times 10^3/\mu\text{L}$	150 - 410
PDW	16.3	fL	9.0 - 14.0
MPV	8.8	fL	7.4 - 10.4
White cell count	2.3	L $\times 10^3/\mu\text{L}$	4.0 - 10.0

Differential white cell Count

	Result	Relative	Ref. Range	Result/cmm	Absolute	Ref. Range
Bands	0	%	Up to 6.00	0.0		
Seg	58.1	%		1.3	$\times 10^3/\mu\text{L}$	2.0 - 7.0
Neutrophils	58.1	%	40.0 - 80.0	1.3	$\times 10^3/\mu\text{L}$	1.0 - 3.0
Lymphocytes	24.9	%	20.0 - 40.0	0.6	$\times 10^3/\mu\text{L}$	Up to 1.0
Monocytes	15.4	%	1.0 - 10.0	0.4	$\times 10^3/\mu\text{L}$	Up to 0.5
Eosinophils	0.8	%	Up to 6.0	0.0	$\times 10^3/\mu\text{L}$	Up to 0.1
Basophils	0.8	%	Up to 1.0	0.0	$\times 10^3/\mu\text{L}$	

Comments :-

Mild macrocytosis. Vitamin B12 or folic acid trial therapy is recommended.
Mild leucopenia with absolute neutropenia and lymphopenia. for clinical correlation.

branch Manager
Dr. Yusra Mohamed
Branch: Nasr city

Verified By
Dr. Amal Fawzy

Unit Manager
Prof. Yousry ElZohairy

Print Date: 22/09/2026 17:25

Page 1 of 1



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